

Core Principles for Systemic Sclerosis Management:

1. Management is organ-specific.
2. New hypertension + AKI: Scleroderma renal crisis → ACE inhibitor (captopril). Avoid steroids.
3. Screen all patients with PFTs (for ILD) and an Echocardiogram (for PAH).
4. Treat Raynaud's with calcium channel blockers (e.g., Amlodipine, Nifedipine). Treat GERD with PPIs.

AVOID: High-dose steroids in systemic sclerosis—they can precipitate renal crisis.

Fibromyalgia

Presentation:

- Chronic, widespread musculoskeletal pain, fatigue, unrefreshing sleep, cognitive "fog".
- Physical exam notable for multiple tender points, but otherwise normal.

Diagnosis:

- **Diagnosis is clinical;** labs/imaging are normal.

PEARL: Fibromyalgia = pain with normal strength/CK. **Polymyositis** = weakness with CK elevation.

Core Principles for Fibromyalgia Management:

1. The cornerstone of therapy is non-pharmacologic: patient education, regular low-impact aerobic exercise, and cognitive behavioral therapy (CBT).
2. First-line medications are low-dose Tricyclic Antidepressants (e.g., Amitriptyline) at bedtime.
3. Second-line options include SNRIs (Duloxetine) or anticonvulsants (Pregabalin).
4. Avoid opioids, NSAIDs, and steroids (often ineffective; pain is central sensitization rather than inflammation).

Crystal-Induced Arthritis (Gout and CPPD)

Presentation:

- **Gout:** Acute, intensely painful monoarthritis, classically the 1st MTP joint (podagra). Triggers: alcohol, red meat, diuretics.
- **CPPD (Pseudogout):** Acute monoarthritis in an older adult, typically affecting the knee or wrist, often post-illness/surgery.

DIAGNOSTIC PATHWAY (ARTHROCENTESIS IS GOLD STANDARD)

Normal serum uric acid during an acute flare does not rule out gout; inflammatory cytokines can increase renal urate excretion.